

Prior Authorization Request

Health Plan

Humana Medicare Advantage -- Gold Choice PPO.

Member Information

Member: Wilhelmina C. Asante DOB: 1960-06-06 (age 66) Member ID: MBR-58172

Sex: F

Requesting Provider

Practice: Clearwater Heart & Vascular

Requested Service

Requested service: Elective inpatient admission, implantation of a transvenous implantable cardioverter defibrillator (ICD)

CPT 33249

Planned admit date: 2026-10-20

Level of care requested: INPATIENT

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General cardiology note

CLEARWATER HEART & VASCULAR

Office note 09/10/2026 Provider: Jonas Whitfield, MD

Pt: Asante, Wilhelmina C. 66F

HFrEF dx 07/2026 (presented w/ edema/DOE). Long-standing DM2 and CKD 4 (eGFR 24).

Ischemic eval: invasive angiography deferred due to CKD 4 and contrast risk; pt also prefers to avoid cath. SPECT 07/30 w/ small fixed inferior defect, read as attenuation vs prior silent infarct.

Impression: Cardiomyopathy, etiology undetermined. I favor ischemic given long-standing diabetes and the fixed inferior defect, but cannot exclude a non-ischemic process.

GDMT started 08/01, tolerating.

Refer to EP re ICD.

J. Whitfield MD

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Nuclear stress test report

CLEARWATER NUCLEAR CARDIOLOGY

Regadenoson SPECT 07/30/2026

ASANTE, WILHELMINA

Perfusion: small, mild-intensity fixed defect of the basal inferior wall. Breast and diaphragmatic attenuation present; attenuation-corrected images partially normalize the defect.

No reversible defects.

Gated LVEF 29% (limited, irregular gating).

Impression: 1) Small fixed basal inferior defect - attenuation artifact vs small prior infarct; cannot exclude. 2) No ischemia. 3) Reduced LVEF.

Read: R. Castellano-Byrd, MD

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Echocardiogram report

CLEARWATER ECHO LAB

Date: 09/17/2026

Asante, Wilhelmina C.

LVEF 27% (biplane Simpson).

Global hypokinesis without clear regional pattern. LV dilated (LVIDd 6.3).

Mod functional MR. RV mildly reduced.

Impression: severe global LV dysfunction, LVEF 27%.

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EP consultation

CLEARWATER HEART & VASCULAR - ELECTROPHYSIOLOGY

Consult 09/24/2026

Pt: Wilhelmina Asante

Sx: SOB walking room to room on bad days, usually half a block; no rest dyspnea. NYHA III.

Echo 9/17: LVEF 27%, global HK, no regional WMA.

No known MI. No PCI or CABG ever.

Assessment: Cardiomyopathy, likely non-ischemic given global pattern on echo; however ischemic etiology is not excluded without angiography (deferred for CKD 4).

On GDMT since 08/01/2026.

Plan: pt and family want ICD before winter travel. SDM visit, then implant 10/20.

Y. Mbatha-Rowe, MD

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Medication list

Asante, Wilhelmina - Current meds - 09/24/2026
carvedilol 6.25 mg BID start 08/01/2026
valsartan 40 mg BID start 08/01/2026 (ARNI not used - CKD 4/K)
dapagliflozin 10 mg start 08/01/2026
hydralazine 25 mg TID start 08/01/2026
isosorbide dinitrate 20 TID ... start 08/01/2026
torsemide 20 mg daily start 07/2026
insulin detemir 22 u qHS 2012
atorvastatin 40 2015

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Shared decision making note

Clearwater Device Clinic - SHARED DECISION MAKING (ICD)

Date: 09/29/2026

Patient: Wilhelmina Asante, with son

Facilitator: Precious Dube, NP

Decision aid: evidence-based ICD decision aid (booklet), reviewed fully.

Discussed that ICD benefit may be smaller with CKD 4; shocks; infection; dialysis-access considerations for venous anatomy; deactivation.

Decision: proceed.

P. Dube NP

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Pre-admission plan

PRE-ADMISSION PLAN 10/02/2026

Asante, Wilhelmina C. MBR-58172 CPT 33249 Admit 10/20/2026

Expected LOS: 2 midnights.

Factors: CKD 4 (eGFR 24) - nephrology to follow, no contrast, daily BMP; insulin-requiring DM2 - glucose mgmt while NPO; preservation of L arm veins for future AV access (R-sided implant).

Limiting conditions screened: none (no dementia, no terminal illness, no AF, stable).

Y. Mbatha-Rowe MD

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